Prior Authorization Request Packet

Case pa_59f7de04 · Aetna · MRI knee (lower extremity joint) without contrast (73721) · Policy Aetna CPB 0171
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1. Letter of medical necessity

October 11, 2026

Aetna Prior Authorization Department

RE: Marcus Lindqvist, DOB 1991-06-23, Member ID AET-513980027
Requested service: MRI knee (lower extremity joint) without contrast (CPT 73721)
Policy: Aetna CPB 0171, https://www.aetna.com/cpb/medical/data/100_199/0171.html

To the Medical Reviewer:

I am writing on behalf of the ordering physician, Dr. Priya Raman, to request authorization for a right knee MRI without contrast for Mr. Lindqvist, who is described in the note as a "35-year-old male recreational soccer player" with "persistent right knee pain and intermittent swelling".

Clinical summary: The note states, "He recalls a twisting injury during a game earlier this summer." It continues, "Since then he has had medial knee pain, swelling after activity, and a sensation of the knee giving way on stairs." On examination the note records "Moderate right knee effusion." It also records "Medial joint line tenderness." and "McMurray test positive medially with a palpable click." It documents "Lachman negative, posterior drawer negative." and "Range of motion 5 to 120 degrees, limited by pain." The assessment is "Suspected right medial meniscus tear with persistent effusion and instability."

Criteria review under CPB 0171:

Criterion 1, persistent knee pain, swelling, and/or instability: Met. The note documents "persistent right knee pain and intermittent swelling" and describes giving way on stairs, as quoted above.

Criterion 2, no response to at least 3 weeks of conservative therapy: Met. The note states, "He has been doing physical therapy and taking ibuprofen, and has been using a brace, but the symptoms have persisted." The note does not give a duration. Per telephone confirmation with the ordering physician, the duration of physical therapy was stated as "it's been five, six weeks, uh, that the patient has been on the physical therapy". The call date is not recorded in the verified facts. This is about 5.5 weeks against the 3 required.

Criterion 3, multi-view x-rays ruling out fracture or loose body: Met. The note states, "Right knee x-rays, 3 views, obtained 09/15/2026: no fracture, no loose body, joint spaces preserved, small effusion."

The policy also lists separate indications. Persistent true locking of the knee is not documented as present. The note states, "He denies true locking of the knee." I am not relying on this indication. Suspected osteomyelitis or septic joint is not documented as a concern, and the note records "No fever, no redness." and "No warmth or erythema." Tumor evaluation is not the basis of this request, and no facts in the record address it.

Conclusion: Based on the documentation, the request meets the three criteria above: persistent symptoms, more than 3 weeks of conservative therapy without adequate response, and multi-view radiographs excluding fracture and loose body. The request does not rest on the locking, infection, or tumor indications. The note states the plan to "Continue physical therapy pending imaging." We respectfully request approval of CPT 73721.

Sincerely,

Priya Raman, MD
NPI 1891234567

2. Criteria checklist with verified evidence

MET aetna-73721-c1: Persistent knee pain, swelling, and/or instability (giving way).
"persistent right knee pain and intermittent swelling"

MET aetna-73721-c2: Not responding to at least 3 weeks of conservative therapy (conservative therapy consists of a combination of rest, ice, compression, elevation, plus NSAIDs and/or physical therapy).
"it's been five, six weeks, uh, that the patient has been on the physical therapy" [phone_call]
code check: 5.5 weeks documented vs 3 required

MET aetna-73721-c3: Multi-view x-rays have been obtained and have ruled out a fracture or loose body in the knee.
"Right knee x-rays, 3 views, obtained 09/15/2026: no fracture, no loose body, joint spaces preserved, small effusion."

MISSING aetna-73721-r1: Persistent true locking of the knee indicative of a torn meniscus or loose body (more than a momentary locking of the joint with the knee in a flexed position).

MISSING aetna-73721-r2: Suspected bone infection (i.e., osteomyelitis) or septic joint.

MISSING aetna-73721-r3: Detection, staging, and post-treatment evaluation of tumor of the knee.

3. Source visit note

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CHIEF COMPLAINT: Right knee pain and swelling.

HISTORY OF PRESENT ILLNESS: 35-year-old male recreational soccer player presents with
persistent right knee pain and intermittent swelling. He recalls a twisting injury during a
game earlier this summer. Since then he has had medial knee pain, swelling after activity,
and a sensation of the knee giving way on stairs. He denies true locking of the knee. No
fever, no redness. He has been doing physical therapy and taking ibuprofen, and has been
using a brace, but the symptoms have persisted.

PHYSICAL EXAM: Moderate right knee effusion. Medial joint line tenderness. McMurray test
positive medially with a palpable click. Lachman negative, posterior drawer negative. Range
of motion 5 to 120 degrees, limited by pain. No warmth or erythema.

IMAGING: Right knee x-rays, 3 views, obtained 09/15/2026: no fracture, no loose body, joint
spaces preserved, small effusion.

ASSESSMENT: Suspected right medial meniscus tear with persistent effusion and instability.

PLAN: MRI right knee without contrast (CPT 73721) to evaluate meniscus and ligaments.
Continue physical therapy pending imaging.
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4. Telephone confirmation with ordering physician

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agent: Hi, this is the prior-authorization assistant from the clinic. Am I speaking with Dr. Priya Raman?
physician: Yes. I am.
agent: Thanks, Dr. Raman.

I’m calling about your patient Marcus Lindqvist. Aetna requires prior authorization for an MRI knee (lower extremity joint) without contrast, and under Aetna CPB 0171 the visit note is missing one detail. This should only take a minute.

First question:
How many weeks has he been on physical therapy and ibuprofen, and during that period was he also using rest, ice, compression, and elevation?
physician: So I think, uh, it's been five, six weeks, uh, that the patient has been on the physical therapy. And during that time, he wasn't using the eyes and any compression, the rest?
physician: And the and the elevation during the time was yeah. It was good enough.
agent: Got it, so about five to six weeks of physical therapy, with ibuprofen, and he has mainly been using rest and elevation, but not really ice or compression. Thank you, that’s everything I needed. We’ll finish the packet and submit it today. Have a good day.